Harrington Physical Therapy

615 Finley Road Suite 809
Pasadena, TX
Phone 281-555-4316

Itemized Medical Bill

Patient:	Tyler King
Date of birth:	October 10, 1996
Insurance claim:	C-1255
Date of injury:	April 28, 2024
First visit:	May 2, 2024
Last visit:	July 19, 2024
Number of visits:	26

Services

Service	Units	Charge
99203 New patient evaluation	1	$1,024.52
98941 Spinal manipulation	26	$3,278.25
97110 Therapeutic exercise	26	$2,878.52
97014 Electrical stimulation	13	$1,509.87
TOTAL		$8,691.16

Harrington Physical Therapy

615 Finley Road Suite 809
Pasadena, TX
Phone 281-555-4316

Treatment Summary

Patient:	Tyler King
Treating provider:	Kenneth Potter, DC
Diagnosis:	neck and back strain

Patient presented following a mootr vehicle collision with complaints of cervical and lumbar strain, including neck stiffness, reduced range of motion, paraspinal muscle tenderness, and intermittent low back pain radiating into the gluteal region without neurological deficit. Initial examination revealed restricted cervical rotation and flexion, along with palpable muscle guarding in the upper trapezius and lumbar paraspinal musculature. Neurological screening was within normal limits.

Treatment consisted of a standard plan of chiropractic spinal adjustments directed at the cervical and lumbar segments, combiend with soft tissue mobilization. A progressive therapeutic exercise program focused on postural correction, core stabilization, and cervical range of motion was introduced and advanced as tolerated. Electrical stimulation was applied to the affected paraspinal muscles for pain modulation and muscle relaxation during the active treatment phase.

The patient attended a total of 26 visits over the course of care. Progress was documented at each session with reassessment of range of motion, pain scores, and functional tolerance. Subjective and objective findings improved steadily and have now plateaued. The patient has reached maximum medical improvement, with residual symptoms that are stable and not expected to change significantly with further conservative care. Discharge to a independent home exercise program is recommended, with follow-up as needed for flare-ups.